Tommy Trojan Hospital

Request for Proposal (RFP): Picture Archiving and Communication System (PACS)

RFP Number	TT-PACS-2025
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Purpose. Tommy Trojan Hospital is soliciting proposals from qualified vendors to provide a Picture Archiving and Communication System (PACS) to support enterprise-wide medical imaging, image distribution, and long-term archiving.

Note to vendors (for homework, this is descriptive only): Submission instructions and evaluation criteria are provided in Section 8.

1 Introduction and Background

Tommy Trojan Hospital is issuing this Request for Proposal (RFP) to solicit comprehensive vendor responses for the acquisition, deployment, and long-term support of an enterprise-grade Picture Archiving and Communication System (PACS). The selected solution must support a fully digital imaging ecosystem that enhances clinical workflow efficiency, diagnostic accuracy, image availability, and system interoperability across the organization.

The goal of this procurement is to implement a PACS that aligns with industry best practices and current standards (DICOM, HL7/FHIR, IHE integration profiles), ensures optimal system uptime and rapid image delivery, and positions Tommy Trojan Hospital for future advancements in enterprise imaging, artificial intelligence (AI), and data-driven clinical decision support.

1.1 Hospital Overview

Tommy Trojan Hospital is a 200-bed, high-profile acute care hospital serving the Greater Los Angeles metropolitan area. The institution provides comprehensive inpatient, outpatient, and emergency medical services and maintains referral relationships with major health systems throughout Southern California, including UCLA Health and Cedars-Sinai. To remain competitive and clinically distinguished, Tommy Trojan Hospital aims to leverage advanced imaging informatics infrastructure that supports rapid turnaround, cross-departmental access, and expansion into remote and ambulatory care settings.

The Radiology Department performs approximately 185,500 imaging examinations per year across all major diagnostic modalities, with an anticipated annual growth rate of 3%. Imaging services support varied clinical domains, including trauma, cancer care, cardiovascular medicine, neurology, women's health, and primary care. Ensuring consistent access to imaging data throughout the enterprise — including affiliated physician offices — is essential for continuity of care.

1.2 Clinical and Operational Objectives

Through this RFP, Tommy Trojan Hospital seeks proposals for a PACS that achieves the following clinical and operational objectives:

- Provide a unified platform for acquisition, storage, distribution, and viewing of diagnostic images across all modalities.
- Improve radiologist efficiency through advanced visualization, optimized worklists, and intelligent hanging protocols.
- Reduce report turnaround time by enabling rapid access to prior studies and seamless integration with the hospital's RIS/EHR.
- Expand image access to offsite physician practices and clinical partners through secure web-based and mobile viewing solutions.
- Standardize workflow across modalities and technologist teams to reduce variability and improve operational throughput.

1.3 Technical and Strategic Objectives

Through this RFP, Tommy Trojan Hospital seeks proposals for a PACS that achieves the following technical and strategic objectives:

- Replace or augment existing imaging systems with a standards-compliant, scalable platform.
- Support short-term storage (12 months) and long-term retention (7 years) consistent with institutional policy and regulatory requirements.
- Maintain high availability through robust fault tolerance, disaster recovery, and cybersecurity practices.
- Prepare the organization for emerging technologies, including, but not limited to:
  - AI-driven decision support
  - Cloud-based image lifecycle management
  - Vendor Neutral Archives (VNA)
  - Enterprise imaging analytics

1.4 RFP Governance and Vendor Expectations

Vendors responding to this RFP are expected to provide a detailed, structured response addressing each section of the document. The hospital will evaluate proposals based on compliance with functional and technical requirements, total cost of ownership, implementation readiness, demonstrated experience in similar deployments, and long-term service capabilities.

No award will be made solely on price; technical merit, scalability, reliability, workflow fit, and service quality are critical determinants.

2 Scope of Work

The selected vendor will deliver a fully integrated PACS solution that supports enterprise-wide diagnostic imaging and clinical review. The solution must encompass acquisition, storage, workflow orchestration, distribution, visualization, lifecycle management, and long-term archival.

The vendor is responsible for providing all required hardware, software, interfaces, cybersecurity safeguards, services, and documentation necessary to configure, validate, deploy, and support the PACS environment.

2.1 Clinical and Operational Scope

The PACS must support:

1. All modalities currently in use at Tommy Trojan Hospital (CR, DR, CT, MR, US, NM, PET, mammography, and angiography).

2. All clinical service areas, including inpatient, outpatient, operating rooms, emergency department, trauma, intensive care, and offsite clinics.
3. Concurrent use by radiologists, technologists, clinicians, and advanced practitioners.
4. Secure remote access for affiliated providers across the metropolitan region.

The solution must maintain compatibility with existing hospital systems (e.g., RIS, EHR, ADT, reporting tools) and allow for interoperability enhancements as future needs evolve.

2.2 Technical Scope

The PACS must provide:

- Multi-modality DICOM ingestion, storage, prefetching, and retrieval.
- Integration with RIS/EHR through HL7/FHIR interfaces and IHE profiles (e.g., SWF, PIR, CPI, ATNA).
- Diagnostic and enterprise viewers with advanced visualization tools.
- Encryption, audit logging, and role-based access control to satisfy HIPAA and institutional security requirements.

2.3 Implementation Scope

Vendor responsibilities include workflow analysis, configuration, testing, training, go-live support, data migration, and post-deployment optimization.

2.4 Support and Maintenance

Vendors must provide 24/7 support, defined SLAs, software updates, security patches, system monitoring, and lifecycle management.

3 Current Imaging Environment and Exam Volumes

Tommy Trojan Hospital conducts approximately 185,500 imaging examinations annually across CR, DR, CT, MR, ultrasound, nuclear medicine, PET, and mammography. Annual volume is projected to increase at a rate of 3% per year. Current workflows are fragmented, with inconsistent access to prior studies, limited enterprise visibility, and variable retrieval times that impact report turnaround.

3.1 Existing Modalities and Annual Volume

Table 1 summarizes the current annual imaging exam volume by modality for Tommy Trojan Hospital. These values are based on the Procedure Volume Analysis provided in the assignment.

Modality	Exams/year	Images/exam	MB/image	Annual growth
CR portables	35000	4	8.0	3%
CR other	75000	4	8.0	3%
CT multi-row avg	7500	300	0.5	3%
MR	15000	96	0.5	3%
Ultrasound (static)	30000	36	0.9	3%
Nuclear	13000	12	0.06	3%
PET	5000	40	0.13	3%
Total	185500	–	–	3%

Table 1: Annual exam volume and imaging parameters by modality.

Modality Mix and Percentage Distribution

Calculation. *Using the modality exam counts in Table 1, we compute the exam-volume percentage distribution for each imaging type. Let E_i denote the annual exam count for modality i, and $E_{tot} = 185{,}500$ the total yearly volume. Then,*

$$p_i = \frac{E_i}{E_{tot}} \times 100\%.$$

Thus, $p_{CR-port} = \frac{35{,}000}{185{,}500} \approx 18.9\%$,
$p_{CR-other} = \frac{75{,}000}{185{,}500} \approx 40.4\%$,
$p_{CT} = \frac{7{,}500}{185{,}500} \approx 4.0\%$,
$p_{MR} = \frac{15{,}000}{185{,}500} \approx 8.1\%$,
$p_{US} = \frac{30{,}000}{185{,}500} \approx 16.2\%$,
$p_{NM} = \frac{13{,}000}{185{,}500} \approx 7.0\%$,
$p_{PET} = \frac{5{,}000}{185{,}500} \approx 2.7\%$.

These percentages describe the modality mix that the PACS must support for load distribution, system sizing, caching strategy, and future architectural planning.

To model projected modality-specific growth, we apply the 3% annual increase:

$$E_i(t) = E_i(0) \times (1.03)^t,$$

yielding a seven-year forward projection used for archive planning and network throughput estimation.

Storage Sizing Calculations

Calculation. *We compute the annual imaging data volume based on the procedure volume analysis (PVA) table. For each modality i, let:*

$$D_i = (exams_i) \times (images/exam_i) \times (MB/image_i).$$

We compute each modality's total yearly data:

$D_{CR-port} = 35{,}000 \times 4 \times 8 = 1{,}120{,}000\ MB,$
$D_{CR-other} = 75{,}000 \times 4 \times 8 = 2{,}400{,}000\ MB,$
$D_{CT} = 7{,}500 \times 300 \times 0.5 = 1{,}125{,}000\ MB,$
$D_{MR} = 15{,}000 \times 96 \times 0.5 = 720{,}000\ MB,$
$D_{US} = 30{,}000 \times 36 \times 0.9 = 972{,}000\ MB,$
$D_{NM} = 13{,}000 \times 12 \times 0.06 = 9{,}360\ MB,$
$D_{PET} = 5{,}000 \times 40 \times 0.13 = 26{,}000\ MB.$

Summing:

$D_{total} = 1{,}120{,}000 + 2{,}400{,}000 + 1{,}125{,}000 + 720{,}000 + 972{,}000 + 9{,}360 + 26{,}000 = 6{,}372{,}360\ MB.$

Convert MB → TB (1 $TB = 1{,}048{,}576\ MB$):

$$AnnualVolume = \frac{6{,}372{,}360}{1{,}048{,}576} \approx 6.08\ TB/year.$$

Since Hospital policy requires prefetching one prior per new exam, online storage must hold roughly:

$$OnlineCapacity \approx 2 \times 6.08 = 12.16\ TB.$$

For the 7-year long-term archive:

$$ArchiveRequirement = 7 \times 6.08 = 42.56\ TB.$$

Thus, minimum storage requirements are:

$$Online(12 - month) : 12.2\ TB, \qquad Long - term(7 - year) : 42.6\ TB.$$

3.2 Baseline Workflow Assessment

Scheduling and Ordering. Orders are generated through the EHR/RIS, with partial reliance on manual verification. Prior study retrieval is not consistently automated.

Image Acquisition. Modalities produce DICOM datasets of varying size and complexity. Variability in post-processing contributes to inconsistent study availability.

Radiologist Interpretation. Diagnostic workstations lack standardized hanging protocols. Prior loading may require several minutes under current infrastructure. Reporting integration is limited.

Image Distribution. Enterprise-wide image access is constrained, and zero-footprint viewing is not universally available.

Workflow Improvement Targets

Rationale. *To address current workflow inefficiencies, the Hospital establishes the following measurable objectives for the future-state PACS:*

- *Reduce median CT and MR report turnaround time by* at least 20% *through automated prefetching and standardized radiologist worklists.*

- *Achieve* 95% prior-availability *at the moment a study is first opened by a radiologist.*
- *Reduce technologist routing errors by* 50% *through DICOM MWL and MPPS reconciliation.*
- *Enable enterprise-wide clinician viewing with sub-second access for common modalities (CR, DR, US).*

These objectives form the basis for evaluating vendor proposals and ensuring that workflow modernization directly addresses the Hospital's operational goals.

3.3 Workflow Gaps and Operational Constraints

The Hospital has identified key deficiencies: delayed prior retrieval, limited remote access, fragmented radiologist worklists, inconsistent lifecycle management, and inconsistent security and audit controls.

3.4 Desired Future-State Workflow

The future-state PACS workflow shall provide unified access to current and prior studies, rule-based prefetching, standardized radiologist worklists, secure enterprise-wide viewing, robust integration with RIS/EHR, and continuous availability for diagnostic operations.

4 Functional Requirements

4.1 General System Requirements

The Vendor shall provide an enterprise PACS solution that:

- Supports all required DICOM SOP Classes for CR, DR, CT, MR, US, NM, PET, mammography, and angiography.
- Maintains a minimum of 99.9% uptime excluding approved maintenance.
- Scales to support 3% annual growth.
- Complies with DICOM, HL7, FHIR, and IHE SWF, PIR, CPI, ATNA profiles.

4.2 Image Acquisition and Ingestion

The System shall:

- Automatically ingest all DICOM studies without manual steps.
- Validate demographic and study metadata through HL7/FHIR integration.
- Support modality-specific routing and acquisition workflows.
- Resolve demographic inconsistencies via IHE PIR.

4.3 Radiologist Workflow Requirements

The Vendor shall provide diagnostic workstations that:

- Offer customizable hanging protocols across modalities.
- Support MPR, MIP, and 3D visualization where clinically relevant.
- Integrate or interoperate with reporting tools.
- Load current and prior studies with sub-second performance under typical load.
- Support structured reporting and key image flagging.

4.4 Enterprise Viewing Requirements

The System shall:

- Provide a zero-footprint, browser-based viewer.
- Ensure secure, role-based access control and audit logging.
- Support desktop and mobile platforms without proprietary installation.
- Enable EHR-embedded viewing where possible.

4.5 Workflow Orchestration and Study Management

The Vendor shall provide:

- Centralized radiologist worklists with prioritization and filtering.
- Real-time study status updates integrated with RIS/EHR.
- Configurable alerts for critical results and unread studies.

4.6 Storage, Prefetching, and Lifecycle Management

The System shall:

- Support tiered storage with rule-based lifecycle policies.
- Prefetch one prior exam per new exam.
- Maintain 12 months online storage and 7 years long-term retention.
- Ensure retrieval times meet diagnostic workflow requirements.
- Implement redundant failover mechanisms.

4.7 Innovation and Future Capabilities

The Vendor shall describe capabilities for:

- AI-assisted workflow enhancements.
- Cloud or hybrid architectures and VNA integration.
- Imaging analytics dashboards.
- APIs for future advanced imaging applications.

Hospital-Specific Innovation Requirements

Rationale. *Given the Hospital's service profile, including high-volume trauma, oncology, and cardiology programs, the Vendor shall describe how advanced technologies such as AI triage, oncology follow-up change-detection, and cardiac imaging analytics will be integrated into the PACS workflow. These capabilities must operate without disrupting radiologist reading patterns and shall enhance care quality by accelerating detection of critical findings and improving consistency in longitudinal comparisons.*

5 Technical Requirements and Storage Architecture

5.1 System Architecture Requirements

The Vendor shall provide a PACS architecture that:

- Supports enterprise scalability for increasing study volume and user concurrency.
- Employs modular, service-oriented components to enable independent scaling.
- Maintains 99.9% uptime excluding approved maintenance windows.
- Provides redundancy across application, database, archive, and network layers.
- Supports multi-site routing and caching for affiliated offices.
- Optimizes bandwidth usage through compression, streaming, and caching.

5.2 Standards Compliance and Interoperability

The System shall:

- Support all relevant DICOM Storage and Query/Retrieve SOP Classes.
- Support DICOM MWL, MPPS, and DICOMweb (WADO-RS, QIDO-RS, STOW-RS).
- Support HL7 v2.x ADT/ORM/ORU messaging and optional FHIR R4 APIs.
- Comply with IHE profiles: SWF, PIR, CPI, ATNA, and XDS-I.b (preferred).

5.3 Performance Requirements

The System shall:

- Make new studies available within 60 seconds of receipt.
- Retrieve online priors within 2–5 seconds depending on study type.
- Support at least 200 concurrent clinical viewing sessions.
- Implement transport optimization via compression, caching, and streaming.

5.4 Network Architecture Requirements

Rationale. *To ensure reliable image transmission, concurrent viewing, and future growth, the Vendor shall design the PACS to operate within an enterprise-grade network architecture. The following requirements shall apply:*

- ***Core Network Bandwidth.*** *The PACS shall support operation over a redundant 10 GbE (minimum) core network backbone within the data center. For institutions with imaging research or advanced visualization workloads, 40 GbE uplinks shall be supported.*
- ***Modality-Segment Bandwidth.*** *Acquisition modalities shall be connected to at least 1 GbE interfaces, with CT, MR, PET, and high-throughput CR/DR rooms prioritized for multi-gigabit uplinks where available.*
- ***Network Segmentation.*** *PACS services shall operate on segmented VLANs separating: (1) modality traffic, (2) workstation/viewer traffic, (3) archive/DB traffic, and (4) administrative interfaces, consistent with security best practices.*
- ***WAN / Remote-Site Connectivity.*** *The PACS shall accommodate operation over hospital-connected WAN links with minimum sustained throughput of 100 Mbps per remote clinic. Streaming and progressive rendering shall mitigate the effects of constrained WAN bandwidth.*
- ***Latency Tolerance.*** *For diagnostic reading workstations, end-to-end latency (modality-to-viewer) should not exceed 40 ms under normal operating conditions. The Vendor shall document expected performance under 80–100 ms latency scenarios for remote locations.*
- ***Network Redundancy.*** *Switch-level, link-level, and routing redundancy shall ensure that loss of any single network component does not degrade viewer performance or study ingestion.*
- ***Network Growth Modeling.*** *The Vendor shall account for projected 3% annual imaging growth and the modality-specific percentage distribution computed in Section 3, ensuring that routing, caching, and archive retrieval performance remain within SLA targets for the 7-year planning horizon.*

This network architecture establishes the foundation for predictable PACS performance, scalability, and resilience under enterprise clinical loads.

5.5 Security, Compliance, and Audit Requirements

The Vendor shall ensure:

- Encryption of all data in transit (TLS 1.2+) and at rest (AES-256).
- Role-based access control and MFA for remote users.
- Comprehensive audit logging for access, authentication, and configuration events.
- Documented patch management and cybersecurity hardening.

5.6 Disaster Recovery and Business Continuity

The System shall:

- Include redundant archives in separate physical locations.
- Provide an RTO of under 10 minutes for online studies.
- Ensure zero study loss for successfully transmitted exams.
- Replicate metadata and images across archives automatically.

5.7 Storage Architecture and Capacity Planning

Quantitative Storage Requirements

Calculation. *Using the calculations in Section 3, the annual imaging data volume is approximately:*

$$6.08\ TB/year.$$

Hospital policy requires the ability to store one full year of data online as well as one prior per incoming study for prefetching. Therefore the minimum online storage requirement is:

$$OnlineStorage(12months) \approx 12.2\ TB.$$

With a mandated seven-year retention requirement, the long-term archive must accommodate:

$$Long-termArchive \approx 42.6\ TB.$$

These thresholds represent baseline requirements to be exceeded by Vendor proposals when accounting for metadata overhead, RAID redundancy, and future growth at 3% annually.

The System shall:

- Maintain 12 months of online storage for immediate access.
- Prefetch one prior study for every new study.
- Provide long-term storage for 7 years with cost-efficient storage tiers.
- Retrieve long-term studies within 45–90 seconds depending on study size.
- Implement redundant storage and failover mechanisms.

Network–Storage Interaction Consideration

Rationale. *Since image retrieval performance depends jointly on storage architecture and network throughput, the Vendor shall ensure that the quantitative storage capacity derived earlier is matched with sufficient network bandwidth, segmentation, and latency tolerance as defined in Section 5.4. The PACS must maintain SLA performance levels under the combined effects of:*

- *annual imaging volume growth,*
- *modality-specific workload distribution,*
- *remote-site WAN variability,*
- *and concurrent-viewing load.*

5.8 Data Migration Requirements

The Vendor shall:

- Provide a comprehensive data migration plan including validation steps.
- Maintain referential integrity and ensure zero data loss.
- Supply tools for DICOM tag validation and duplicate detection.

5.9 System Administration and Monitoring Requirements

The System shall:

- Provide secure administrative consoles for configuration and audit.
- Include monitoring dashboards for system health and modality connectivity.
- Generate automated alerts for failures, thresholds, and security events.

5.10 Documentation and Technical Deliverables

The Vendor shall provide:

- Architecture diagrams, data flows, interface specifications.
- Installation, configuration, and operation manuals.
- Disaster recovery and security hardening guides.

6 Implementation Plan and Timeline

The Vendor shall provide a comprehensive implementation plan that enables the successful deployment of the PACS across all clinical areas of Tommy Trojan Hospital. The implementation shall be executed in five phases, each with defined deliverables and acceptance criteria.

6.1 Phase 1: Project Initiation and Requirements Validation

The Vendor shall:

- Assign a project manager and implementation team.
- Conduct a kickoff meeting and workflow discovery sessions.
- Deliver a Project Implementation Plan (PIP) including timelines, resource assignments, testing strategy, and risk mitigation.
- Document interface requirements and provide specifications.

Deliverables: Requirements Traceability Matrix, workflow documents, final PIP. **Acceptance Criterion:** Approval from Hospital project leadership.

6.2 Phase 2: System Installation, Integration, and Configuration

The Vendor shall:

- Install PACS components including image manager, archive, routing services, viewers, and administrative tools.
- Configure DICOM and HL7/FHIR interfaces.
- Implement authentication, RBAC, and storage lifecycle policies.
- Configure prefetching and modality connectivity.

Deliverables: Configuration documentation, interface test plans, connectivity matrix. **Acceptance Criterion:** Successful interface and connectivity tests.

6.3 Phase 3: Data Migration, Testing, and Clinical Validation

The Vendor shall:

- Assess source archives and develop a migration strategy.
- Execute data extraction, validation, transfer, and reconciliation.
- Conduct unit, integration, workflow simulation, and user acceptance testing (UAT).
- Demonstrate reliable performance, correct routing, and metadata integrity.

Deliverables: Test scripts and results, UAT sign-off, migration verification report. **Acceptance Criterion:** Formal UAT approval.

6.4 Phase 4: Training, Go-Live Preparation, and Cutover

The Vendor shall:

- Provide training for radiologists, technologists, clinicians, and IT administrators.
- Supply training materials and quick-reference guides.
- Conduct go-live readiness assessments.
- Develop a Cutover Plan with backout procedures.

Deliverables: Training reports, Cutover Plan, readiness validation. **Acceptance Criterion:** Hospital approval to proceed with go-live.

6.5 Phase 5: Go-Live, Stabilization, and Optimization

During go-live, the Vendor shall:

- Provide onsite or virtual at-the-elbow support.
- Monitor performance continuously and resolve issues proactively.
- Conduct daily status reviews during stabilization (minimum 14 days).

Within 60 days post-go-live, the Vendor shall:

- Optimize hanging protocols, routing paths, and storage policies.
- Deliver a post-implementation optimization report.

Deliverables: Support logs, optimization report, updated configuration documents. **Acceptance Criterion:** Hospital acceptance of optimization outcomes.

6.6 High-Level Timeline

Phase	Description	Estimated Duration
1	Project Initiation & Requirements Validation	4–6 weeks
2	Installation, Integration, Configuration	6–8 weeks
3	Migration, Testing, Clinical Validation	6–10 weeks
4	Training & Go-Live Preparation	3–4 weeks
5	Go-Live & Stabilization	2–4 weeks
Total Estimated Duration		18–28 weeks

The Vendor may propose alternative timelines with justification.

7 Vendor Qualifications, Service, and Support Requirements

7.1 Vendor Qualifications and Experience

The Vendor shall:

- Have at least five (5) years of experience delivering PACS solutions to acute care hospitals or large imaging networks.
- Provide evidence of three (3) comparable implementations of similar scale (150,000 annual studies) within the last five years.
- Demonstrate expertise in multi-modality workflows, IHE integration profiles, enterprise viewing, and tiered archive management.
- Provide indicators of financial stability sufficient for long-term product viability.
- Demonstrate knowledge of HIPAA, HITECH, CMS imaging regulations, and applicable state requirements.

7.2 References and Client Testimonials

The Vendor shall provide three (3) references from comparable healthcare organizations, including contact information for PACS administrators, IT leadership, and radiology department stakeholders. The Hospital reserves the right to contact additional Vendor clients.

7.3 Account Management and Staffing Requirements

The Vendor shall:

- Assign a dedicated Account Manager and certified Project Manager.
- Provide an implementation team including PACS specialists, interface engineers, storage/archiving experts, training staff, and technical support analysts.
- Maintain staffing continuity throughout the project.

7.4 Technical Support Requirements

The Vendor shall provide:

- 24/7/365 support for critical issues.
- Multi-channel access (phone, portal, email).
- Defined escalation pathways (L1–L4) with response and communication protocols.

Severity	Description	Response	Resolution
Critical (P1)	System down	$\leq$ 15 min	$\leq$ 4 hours or workaround
High (P2)	Workflow impacted	$\leq$ 30 min	$\leq$ 12 hours
Medium (P3)	Usability impacted	$\leq$ 1 hour	$\leq$ 5 business days
Low (P4)	Enhancement/minor issue	$\leq$ 1 day	Release dependent

Incident Response SLAs

7.5 Software Updates and Upgrade Policy

The Vendor shall:

- Provide all minor and major software releases during the contract term.
- Ensure backward compatibility of upgrades.
- Conduct upgrade testing in a staging environment.
- Provide detailed release notes and updated documentation.

7.6 Knowledge Transfer and Documentation

The Vendor shall:

- Provide complete technical documentation including architecture, configuration, storage policies, and interface specifications.
- Deliver training and knowledge transfer sessions for PACS administrators, IT staff, radiologists, and technologists.
- Update documentation within 30 days of major releases.

7.7 Long-Term Partnership Requirements

The Vendor shall:

- Participate in quarterly business reviews assessing performance, SLA compliance, and roadmap alignment.
- Maintain compatibility with emerging standards (DICOMweb, FHIR, IHE).
- Provide assurance of long-term support for deployed components.

8 Proposal Format, Vendor Response Structure, and Submission Requirements

8.1 General Submission Requirements

The Vendor shall submit a complete proposal including a written response to all sections of this RFP, a completed Requirements Compliance Matrix, and all supporting documentation. Proposals shall be formatted in PDF, written in English, paginated, and submitted electronically.

Vendor Response Completeness Requirement

Note. *To ensure evaluative fairness, Vendors shall substantiate all claimed qualifications in Section 7 through references, documentation, certifications, or attestation statements. Any omission may result in a finding of non-responsiveness.*

8.2 Proposal Submission Format

Each proposal shall be organized in the following order:

8.2.1 Cover Letter

The Vendor shall identify its legal name, authorized representative, proposal validity period (minimum 120 days), and acknowledgment of RFP addenda.

8.2.2 Executive Summary

The Vendor shall summarize the proposed PACS solution, key differentiators, implementation strategy, and alignment with Hospital objectives.

8.2.3 Response to RFP Sections 1–7

The Vendor shall mirror the RFP structure. For each requirement, the Vendor shall indicate:

- Fully Meets
- Partially Meets
- Does Not Meet
- Alternative Solution Provided

Explanations are required for any response other than “Fully Meets.”

Compliance and Evaluative Fairness

Note. *Proposals must adhere strictly to the response format defined in this section. Failure to respond to each requirement using the mandated structure (Fully Meets, Partially Meets, Does Not Meet, Alternative Solution Provided) may result in the proposal being deemed non-responsive. These conditions enable objective, equitable evaluation across all bidders.*

8.2.4 Technical Solution Description

The Vendor shall describe system architecture, interoperability (DICOM, HL7/FHIR, IHE), performance expectations, storage design, and HA/DR strategy, with diagrams included.

8.2.5 Implementation Plan

The Vendor shall provide a detailed project plan including timeline, resources, testing methods, risk management, training, and cutover strategy.

8.2.6 Support and Maintenance Model

The Vendor shall describe support tiers, escalation paths, SLA compliance, patch management, and upgrade processes.

8.2.7 Pricing Proposal

The Vendor shall provide itemized pricing for licensing, hardware/software, implementation services, migration, training, annual maintenance, and optional modules.

8.2.8 Required Appendices

- Appendix A — Requirements Compliance Matrix
- Appendix B — Financial Statements
- Appendix C — Technical Architecture Diagrams
- Appendix D — Sample Training Materials (optional)
- Appendix E — Sample SLAs or Support Agreement
- Appendix F — References and Case Studies

8.3 Proposal Submission Instructions

Proposals shall be submitted electronically to `mlwu@andrew.cmu.edu` by December 5, 2025, 11:59 PM. Questions must be submitted no later than five business days prior to this date. Vendors must acknowledge receipt of all addenda.

8.4 Proposal Validity and Conditions

Proposals shall remain valid for 120 days. The Hospital reserves the right to accept or reject proposals, request additional information, negotiate terms, or terminate the RFP process at any time.

8.5 Evaluation Criteria (Informational Only)

Proposals will be evaluated based on:

- Technical compliance and architecture
- Workflow and clinical usability
- Implementation methodology
- Vendor qualifications and references
- Support model and SLAs
- Total cost of ownership
- Long-term strategic fit